14.4 Neurogenic Shock

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • *No specific complaint* **Objective** • Injury to the cervical spinal cord • BP 82/48 mmHg, HR 48/min • Skin warm and dry below the level of injury	**ACTUAL** **Decreased cardiac output** related to loss of sympathetic tone as evidenced by hypotension, bradycardia and warm skin.	MAP within the target the physician has set.	• Keep the spine immobilised. • Give fluids and vasopressors as prescribed. • For symptomatic bradycardia, give atropine as ordered. • Continuous ECG and BP monitoring.	MAP on target: **goal met**.
Subjective • *No specific complaint* **Objective** • Cough weak • Vital capacity reduced	**ACTUAL** **Ineffective breathing pattern** related to respiratory muscle weakness as evidenced by reduced vital capacity.	Ventilation adequate, vital capacity stable and SpO_2 on target.	• Follow vital capacity, SpO_2 and ABG. • If his breathing gets worse, prepare for ventilation. • Assisted coughing, as you were taught.	Ventilation adequate: **goal met**.
Subjective • *No specific complaint* **Objective** • Cannot control body temperature below the injury	**RISK** **Risk for hypothermia** as evidenced by risk factors: loss of sympathetic control below the level of injury.	Core temperature stays at 36–37.5 °C.	• Temperature every 2–4 hours. • Adjust the room temperature and his covers. • Keep him covered during procedures.	Temperature normal: **goal met**.